Staff answers and cover notes

Suggested responses concern the invented learner only. Accept other safe reasoned answers; no diagnosis or judgement of a pupil’s health.

Stress

A response to pressure or change. It can affect the body, thoughts, feelings and actions. A little pressure can help with a task; too much or lasting pressure can make daily life harder.

Sign

A possible clue that someone may feel under pressure. Signs vary and can have other causes. A body map cannot diagnose stress or a condition.

Reframing

Checking a thought against what is known and trying a fairer, more useful way to view the situation. It can include naming a real problem and asking for help.

Invented case key

The learner might be responding to the changed plan. Tense shoulders, a fluttery stomach and poor focus are possible signs; they can have other causes. A fairer thought is, “The order changed. I know parts of these tasks. I can ask for the first step.” The learner can try a gentle strategy and ask staff to make the plan clear.

Body map key

Head/focus: hard to focus. Shoulders/muscles: tense shoulders. Stomach: fluttery stomach. A list, pointing, typed or exact-scribed route is equivalent. Do not ask pupils to locate their own symptoms. Clues can have other causes and do not establish a condition.

Hinge answers

Correct B. If A is chosen, explain that signs alone cannot establish a condition; re-model then use the parallel privately. No public answer round. Correct A. A fairer view can include real support. If unclear, keep the model and revisit later; do not label the learner.

Starter key

The invented claim that stress affects only thoughts is inaccurate. It can affect body, feelings and actions too. No pupil needs to say what stress feels like for them.

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Safe strategy models

Gentle breathing

Sit or stand in your usual supported position. Let an easy breath come in and out at your own pace. Keep breathing comfortable. No hold, forced deep breath, set rate or count. Try briefly, then return to usual breathing. Eyes can stay open. Stop and tell staff if uncomfortable or dizzy. You may observe, use normal breathing or pass. Model: The invented learner pauses and lets a few easy breaths come and go.

Reframing

Use the invented thought: “I will get every part wrong.” What is known? The order changed, but the tasks are familiar. Try: “I know some parts. I can ask for the first step.” Say, point to, type or write the fairer thought. A fairer thought need not be cheerful. It must fit the facts. Do not reframe harm as acceptable or dismiss a real problem. Asking for a clearer plan is valid. Model: “The order changed. I know some parts. I can ask for the first step.”

Accessible movement

With staff agreement, try one small, familiar move that suits your access plan: a gentle hand move, a seated stretch or a few steps in the agreed clear space. Stay in a comfortable range. No touch, exertion, balance challenge or performance target. Use a familiar alternative or observe/pass. Stop if painful or uncomfortable; follow usual health and access plans. Model: The invented learner uses an agreed short hand movement, then asks staff to explain the plan.

Trial records

Offer all three within the timings. Calm practice only: never make a stressful noise, surprise or task demand. Usual health/access plans govern. A pupil may adapt, observe or pass. Record the chosen participation route without inferring effectiveness or mental health. A pupil who cannot use one move may use a familiar accessible option or observe.

Reframing limits

Known fact: the order changed; the tasks are familiar and staff can explain. “I will get every part wrong” is a prediction, not established fact. A fairer thought acknowledges uncertainty and support. Do not use reframing to dismiss harm, discrimination, a real barrier or a need for help.

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Tier guidance and safe evidence

Supported

Use short word choices, story clues, pointing or exact scribe/readback. Match three story signs to map/list. Use a fairer-thought choice and one safe strategy choice. All three trials offered; access/adapt/observe/pass remain.

Independent

Use the story to link three signs, explain a fairer thought and choose a safe strategy with a reason and support. All three trials offered. Usual aids stay.

Aspire

Explain why clues are not a diagnosis. Compare two safe approaches to the invented change, test a practical barrier and keep one final choice with support. All three trials offered; extra reasoning never means more personal disclosure.

Private plan

For the invented learner: when a familiar task order changes, check a fairer thought and ask school staff for the first step. If that is not useful, ask for another explanation or the agreed supervised pause. One useful choice and a support route. No promise to feel calm and no requirement to cope alone. Strategies do not replace help with the cause of pressure. Personal card stays with pupil; a fictional card, blank or pass is valid. Do not collect, photograph, display or compare it.

Self assessment

All boxes start blank. This checks learning, not feelings, symptoms or whether a strategy worked. Observing, adapting or passing is recorded fairly, without a health judgement.

Evidence and marking

Record explanation of stress, three fictional body-map links, a fairer thought, safe strategy knowledge and a generic support route. Record each breathing/reframing/movement opportunity as tried, adapted, observed, passed or not observed; no pass is failure. Do not record symptoms, feelings, private plan content or claimed improvement. Distinguish usual access from actual adult content prompts. Use a work code, one NS and actual pupil response. E requires saved actual work; R requires Voice and Audience plus NS and own response. VF verbal feedback; WS worked with support; I independent; NS next step in green; // self-edit; ? read this back to me; E actual evidence banked; R responded. R only when Voice and Audience are present, one NS is set and the pupil responds in the same colour, on the same page or usual mode. Offer to the Communication portfolio; staff confirm any mapping.

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Safe support and source basis

Discreet supervised pause

Before the lesson, quietly agree the familiar pause signal and supervised place/route with the learner. A pupil may show a pause card or use the usual aid without a public explanation. A staff member acknowledges and supports the agreed route; never send a learner off alone or remove supervision. Offer a neutral task and a supported return, or arrange a later private catch-up. Pass/step-out is not failure and does not require personal disclosure.

Safeguarding and privacy

Any disclosure goes to the safeguarding lead through school procedure. Listen calmly, reassure, do not probe, investigate or promise secrecy. Follow urgent school procedures where needed. Keep actual concerns and identifying details out of the lesson pack, book tasks and shared learning evidence; use the school's separate safeguarding record. If a learner cannot identify a trusted adult, support them privately and do not press. Private plans are not collected, photographed, displayed or compared for assessment. Scribing the personal plan is only by agreement; the learner keeps it.

Help by type

Support may come from a trusted adult, school staff, a family doctor or a suitable helpline. Ask trusted school staff for help to find an age-appropriate route. Seek help if a concern persists, worsens, affects everyday life or feels unsafe; you do not need a diagnosis first. If the first adult is unavailable or unhelpful, try another trusted adult through the agreed route. For immediate concern, get school staff straight away and use the school's urgent procedure.

Source basis

Date, period and exact outcome are from the owner brief, LAUNCH Weekly Spring row 149. The workbook was not reopened. Accepted Week 1 supplies format only. Current official public-health pages on stress, gentle breathing, reframing and young people's movement were checked for narrow factual teaching. Strategies are optional educational practice, not treatment or a clinical assessment. The campaign wording is preserved only as part of the exact outcome.

Where this leads

Next learning applies calm decision-making and help-seeking to safe choices online. Offer fictional learning to the Communication portfolio; staff confirm mapping and evidence sufficiency.

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